

# One connected system for every patient, every visit, every follow-up.

Phase I put a front door on the clinic: a website, WhatsApp booking, and a live queue. Phase II turns that into the record: what was found, what was prescribed, what was billed, and what happens next, all in one place for Dr. Ramachandra and the front desk.

## WHERE THINGS STAND

## What's live today, and the gap it leaves open

01

The front door works. Patients book, get a token, and walk in on time. What happens after they sit down in front of Dr. Ramachandra is still a paper notebook and a memory.

**Live since Phase I**

● Live

Patient-facing website with services, hours and location. WhatsApp booking that issues a real token. A live queue the front desk runs the day from. A lean admin dashboard, with Today, Schedule, and early Patients/Revenue views.

**Still missing**

● Proposed

No consultation note. No structured diagnosis or prescription. No place to store an X-ray or MRI. No invoice beyond a single fee field. No way for a patient to see their own record later. No follow-up ever gets reminded automatically.

**Today, the entire clinical and financial record of a visit is one line** in the appointment table: a free-text reason and a flat fee. Everything Dr. Ramachandra actually decides in the room (the diagnosis, the plan, what was prescribed) lives only in his memory or a paper chit that leaves with the patient.

PATIENTS SEEN / WEEK

**~180**

across morning &amp; evening sessions

RETURN-VISIT RATE

**41%**

fractures, post-op, chronic joint care

RECORDS SEARCHABLE TODAY

**0**

no way to pull a patient's history in seconds

## DESIGN PRINCIPLES

## Why this is the right next step, and how it's being built

02

Phase II isn't a rebuild. It's the same system, taught to remember. Every screen below follows the same three rules Phase I was built on.

1

**Doctor time is the scarce resource**

Every screen is built to be filled in seconds, not minutes. Templates, favourites and autosave do the typing. If it slows the consultation down, it doesn't ship.

2

**Reception stays in control of the room**

Front desk keeps full access to booking, billing and the queue. Only the clinical write actions, diagnosis and prescriptions, are gated to the doctor.

3

**Nothing leaves the clinic without asking**

Patient data stays behind a login. Reports and prescriptions reach the patient only through a signed link or the patient's own portal, never a public URL.

**What "connected" actually means here**

Today, a returning patient's fracture follow-up starts from zero. Dr. Ramachandra re-asks what was already found last visit. With Phase II, opening that patient's file surfaces the last consultation, the X-ray that was uploaded, and the medicines already tried, before he says a word.

The same record that the doctor writes becomes the prescription that prints, the invoice that reception bills, and the update the patient receives on WhatsApp, written once and used four times instead of retyped at every desk.

ONE RECORD, START TO FINISH

What a single visit looks like once Phase II is live

03

01

Book

● Live

Website or WhatsApp. A token and slot are issued instantly.

02

Check in

● Live

Reception marks arrival. Patient joins the live queue.

03

Consult

● New

Doctor opens the patient's history, writes the note.

04

Prescribe

● New

Medicines added from favourites, printed on the spot.

05

Bill

● New

Fee plus any procedures, receipted before they leave.

06

Share

● New

Prescription and receipt land on the patient's portal.

07

Follow up

● New

A WhatsApp reminder goes out on the date the doctor set.

What changes for the patient

They walk out with a printed prescription, a receipt, and a WhatsApp message linking to both, plus a reminder that arrives on its own when it's time to come back. No more losing the paper chit, no more calling to ask "what did the doctor say to take."

What changes for the clinic

Every visit becomes a searchable record. Revenue is billed against real line items, not a flat fee guess. And six months from now, "how many knee replacements did we do this year" is a question the system can answer in seconds.

WORKSTREAM A

Reception dashboard, extended

The Today queue front desk already lives in stays exactly where it is. It just grows a billing action and a real patient directory instead of a beta placeholder.

A

Today · Tue 15 Sep

| TOKEN | PATIENT      | REASON          | STATUS     | ACTION   |
|-------|--------------|-----------------|------------|----------|
| 14    | Ash Kumar    | Fracture review | Consulting | :        |
| 15    | Lakshmi Rao  | Consultation    | Waiting    | :        |
| 13    | Naveen Reddy | Knee follow-up  | Done       | ● Bill → |



**Bill on the spot**  
Once a token is marked done, a "Bill" action opens an invoice pre-filled from the fee plus any consumables the visit used. Reception confirms and prints in one step.



**Real patient search**  
Patients tab becomes a proper search-and-list over every patient ever seen, not just today's appointments, searchable by name, phone, or token history.



**Edge cases handled**  
Walk-ins with no booking, a patient billed twice by mistake (avoidable within the day), and a token called out of order all have a defined, tested path.

WORKSTREAM B · 1 OF 2

The patient record: a real history, not a fresh start every visit

B

Ash Kumar · ROC-002481

42 yrs · O+ · 6 visits

- 12 SEP 2026

**Consultation: fracture review**

Right tibia fracture, 6-week follow-up. Union progressing well on X-ray. Advised weight-bearing to begin gradually.
- 12 SEP 2026

**Report uploaded: X-ray, right tibia**

Uploaded by reception during check-in, linked automatically to today's consultation.
- 12 SEP 2026

**Prescription issued: 3 items**

Calcium supplement, pain relief as needed, physiotherapy referral.
- 1 AUG 2026

**Consultation: cast removal**

Cast removed at 4 weeks. Mild stiffness noted, physiotherapy started.

WORKSTREAM B · 2 OF 2

# Four views into the same patient

The timeline is the front door. Underneath it, four tabs hold the detail so nothing gets buried in a scroll.

B



### Visit history

Every consultation, chronological, with chief complaint and diagnosis visible without opening each one.



### Prescriptions

Every prescription ever issued to this patient, each one re-printable, so a repeat request takes ten seconds.



### Reports

Every X-ray, MRI or lab result, grouped by date, opened via a signed link that expires, never a public file.



### Care plans

Active follow-ups and advice-in-progress for this patient, so nothing scheduled gets forgotten between visits.

### Profile header, at a glance

|                   |               |
|-------------------|---------------|
| Age / blood group | 42 · O+       |
| Allergies         | None recorded |
| Next follow-up    | 26 Sep 2026   |
| Emergency contact | On file       |

### Edge cases handled

Two patients sharing one phone number, a walk-in with no prior record, and a patient who asks to correct their own date of birth all have a defined path: reception edits, doctor never has to.

WORKSTREAM C · 1 OF 2

# The consultation screen, built to disappear

Opens straight from a queued token. Structured, not a blank text box: every field maps to a question Dr. Ramachandra already asks out loud.

C

Consultation · Token 14 · Ash Kumar

Autosaving...

|                 |                          |                |                             |
|-----------------|--------------------------|----------------|-----------------------------|
| Chief complaint | Right knee pain, 3 weeks | Diagnosis      | Early osteoarthritis        |
| History         | No prior injury          | Advice / plan  | Physiotherapy, review 4 wks |
| Examination     | Mild swelling, full ROM  | Follow-up date | 13 Oct 2026                 |



### Left rail, always visible

Today's remaining queue and a shortcut back to this patient's history and reports, with no tab-switching mid-consultation.



### Autosave, debounced

Every field saves quietly a moment after typing stops. Nothing is lost if the doctor is called away mid-note.



### Gated to the doctor

Reception can view a consultation once saved, but only a doctor-role login can write or edit one.



## WORKSTREAM C · 2 OF 2

## Templates: the four visits Dr. Ramachandra sees every day

One tap prefills chief complaint, examination and advice for the clinic's most common cases. He edits from there instead of typing from zero.

C

**Lumbar spondylosis**

Prefills chronic low-back history, standard examination findings, physiotherapy + posture advice, 4-week follow-up.

**Knee osteoarthritis**

Prefills gradual-onset pain history, range-of-motion checks, weight and exercise advice, 6-week follow-up.

**Cervical radiculopathy**

Prefills neck-pain-with-radiation history, neuro exam prompts, referral note, 3-week follow-up.

**Post-fracture review**

Prefills union-check examination, weight-bearing advice, and links straight to the patient's last uploaded X-ray.

Templates are a starting point, never a lock. Every field stays fully editable, and nothing is ever prescribed automatically. **The doctor decides everything; the template just saves him from retyping the routine parts.**

WORKSTREAM C, CONTINUED · 1 OF 2

# The prescription builder

C

Search a medicine, add it as a line, set dosage and duration. Built for the three-to-six line prescriptions this clinic actually writes.

Prescription · Ash Kumar

3 items

| MEDICINE               | DOSAGE | FREQUENCY  | DURATION |
|------------------------|--------|------------|----------|
| Calcium + Vitamin D3   | 500mg  | Once daily | 4 weeks  |
| Diclofenac             | 50mg   | As needed  | 1 week   |
| Physiotherapy referral | :      | 2x / week  | 4 weeks  |

**Favourites, ranked by use**

The medicines Dr. Ramachandra prescribes most often surface first in search, so common items are a tap away, not a full type.

**Reorder, remove, edit inline**

Every line can be reordered, edited or removed before printing, with no separate "delete and re-add" step.

WORKSTREAM C, CONTINUED · 2 OF 2

# Save & print: no new infrastructure required

C

"Save & Print" writes the prescription to the patient's record, then opens a clinic-letterhead page the browser prints or saves as a PDF directly. No server-side PDF rendering to build or maintain.



## One shared print template

The same letterhead layout is reused for prescriptions, invoices and receipts: one visual language, one place to update the clinic's branding.



## Reaches the patient too

The same print page opens from the patient portal, so a lost prescription is a login away instead of a phone call to reception.

TYPICAL BUILD TIME / RX

**<30s**

from favourites + template

NEW INFRA REQUIRED

**Zero**

browser print, no PDF service

REPRINT FROM PATIENT RECORD

**1 click**

any prior prescription

WORKSTREAM D

# Reports, X-rays and MRIs, attached to the visit that ordered them

D

Reception uploads at check-in or the doctor uploads mid-consultation. Either way, the file lands on the patient's timeline, never on a shared drive.



### Upload, tagged by type

X-ray, MRI, lab result or "other," tagged on upload so the patient's Reports tab stays organised without any manual filing.



### Signed links, never public

Files live in a private store. Every view, staff or patient, goes through a link that's minted on request and expires shortly after.



### Linked to the right visit

A report can be tied to the consultation that ordered it, so "the X-ray from the fracture review" is one click, not a search.



### Empty state, handled

A patient with no reports yet sees a clear "nothing uploaded" state with an upload shortcut for reception, never a blank, confusing screen.

WORKSTREAM E · 1 OF 2

# Invoicing that reflects what actually happened

Today, revenue is one flat fee per appointment. Phase II bills line by line: consultation fee, any procedure, any consumable used.

E

Invoice ROC-2026-0148

Paid · UPI

| ITEM                    | AMOUNT |
|-------------------------|--------|
| Consultation: follow-up | ₹400   |
| Plaster removal kit     | ₹150   |
| Total: ₹550             |        |

WORKSTREAM E · 2 OF 2

# Inventory that updates itself when a bill goes out

E

Consumables used in an invoice, casts, dressing kits, splints, decrement stock automatically, so reception isn't counting supplies by hand at month end.

### Stock list

|                        |               |
|------------------------|---------------|
| Plaster of Paris rolls | 32            |
| Crepe bandages         | 18            |
| Splints (adult)        | 4 · low stock |
| Dressing kits          | 61            |

### Every change, logged

A single append-only ledger records every stock movement (a bill going out, a manual restock, a correction) along with who made it and when, so shrinkage is traceable, not guesswork.

### THIS WEEK'S COLLECTIONS

₹24.6k

from invoices, not appointment guess

### LOW-STOCK ITEMS

1

flagged automatically on the dashboard

### PAYMENT METHODS

3

cash, UPI, card

WORKSTREAM F

# A portal that needs nothing but a phone number

No app to install, no password to forget: the same phone-number trust model the booking flow already uses, with a one-time code for anything private.

F

Portal login

Phone number

+91 93... ..203

Code sent via

WhatsApp

Valid for

10 minutes

Hello, Ash

Next follow-up

13 Oct 2026

Latest prescription

View / print

Reports & bills

3 items



Automated reminders

A follow-up date set in a consultation becomes a WhatsApp reminder the day before, no reception call required.



Session, not an account

A signed cookie tied to the verified phone number, with no separate password to create, reset or leak.



Failure states handled

Wrong code, expired code and "phone not on file" each show a clear, specific message, never a silent failure.

WORKSTREAM G · 1 OF 2

What the clinic looked like this week, at a glance



PATIENTS TODAY

**38**

up from 31 last Tuesday

FOLLOW-UPS COMPLETED

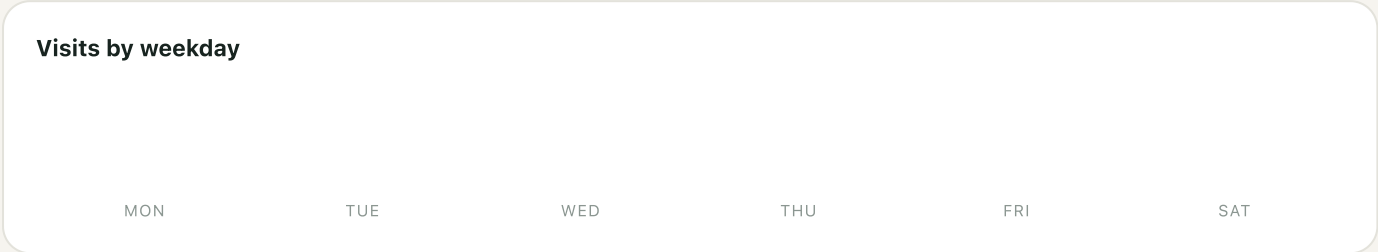
**82%**

of those due this week

COLLECTIONS THIS WEEK

**₹24.6k**

across 61 invoices





WORKSTREAM G · 2 OF 2

# Who can do what, and a record of who did



Every staff login gets a role. Every clinical write is logged. Nobody has to take anyone's word for what happened on a given day.

### Three roles, clearly scoped

|           |                                         |
|-----------|-----------------------------------------|
| Doctor    | Consultations, prescriptions, templates |
| Reception | Booking, billing, queue, inventory      |
| Admin     | Everything, plus staff management       |

### Activity feed

A rolling log of recent actions (who created an invoice, who edited a consultation, who added a staff login) visible to admin, so nothing significant happens invisibly.

A reception login can never write a diagnosis or a prescription, even if they know the screen exists. **The system enforces the boundary, not just the interface.**

## NOT IN THIS PHASE: THE HONEST VERSION

## Where AI could help later, and where it never should

12

**What AI can eventually do**

Draft a consultation summary from the doctor's shorthand for him to review and edit. Flag a patient overdue for a follow-up. Summarise a patient's history into one paragraph before a busy day starts.

**What AI should never do**

Suggest a diagnosis. Choose a medicine or dosage. Contact a patient with anything that reads as medical advice. Every clinical decision stays with Dr. Ramachandra, full stop. This is a boundary, not a limitation to work around later.

## NEW: TIMELINE

## What ships, and when

Ten weeks, in an order that keeps the clinic fully operational at every step, so nothing goes live half-finished.

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**WEEK 1****Foundation**

Data model, staff roles, and the extended patient profile: the plumbing everything else is built on.

**WEEKS 2-3****Doctor workspace & templates**

The consultation screen, autosave, and the four starting diagnosis templates, tested in real consultations.

**WEEK 4****Prescriptions**

Builder, favourites, and the print/PDF page, the first piece patients see directly.

**WEEK 5****Billing**

Line-item invoicing and the updated Revenue view, replacing the flat fee.

**WEEK 6****Reports & imaging**

Upload, signed links, and the private storage bucket wired into the patient timeline.

**WEEK 7****Inventory**

Stock list and the auto-decrement tied to billing.

**WEEK 8****Analytics & staff management**

The weekday chart, follow-up completion, and the admin staff/role screen.

**WEEK 9****Patient portal**

OTP login, the patient dashboard, and prescription/receipt sharing.

**WEEK 10****Follow-up automation & go-live**

The daily reminder job, end-to-end walkthrough with Dr. Ramachandra, and handover.

NEW: PRICING

What Phase II costs

Phase I was priced at ₹75,000 for the front door. Phase II is a bigger build (seven workstreams instead of one), priced the same honest way: one number, two ways to pay it.

14

OPTION A · BUILD, OUTRIGHT

₹2,10,000 one time

- ✓ All 7 workstreams, delivered over 10 weeks
- ✓ 90 days of post-launch support included
- ✓ No monthly commitment either way
- ✓ Clinic owns the build outright from day one

Same infrastructure Phase I already runs on, no new hosting cost.

OPTION B · PHASED, WITH CARE

₹1,30,000 upfront

- ✓ Covers build through week 5 (doctor workspace, prescriptions, billing)
- ✓ Remaining ₹80,000 due at go-live, week 10
- ✓ Ongoing care from ₹3,500/month after launch, same as Phase I's plan
- ✓ Best if cash flow is tighter this quarter

Over the full build, totals the same ₹2,10,000, only the pacing changes.

Both options include everything on the deliverables table on page 23. **Nothing here is priced separately or held back as an "add-on."** WhatsApp message costs from Meta and any future AI usage, if the clinic ever opts in, are the only running costs outside this number.

NEW: APPENDIX · 1 OF 2

# How patient data is actually protected

Written for the clinic, not for engineers: what's true today, and what Phase II adds.

15



### Nothing is public by default

Patient records, prescriptions and reports sit behind row-level security in the database itself. Even if someone had the technical know-how to query it directly, they'd still need to be a recognised staff login to see a single record.



### Staff, not just "logged in"

A login alone proves nothing. Every check also confirms the email is on the clinic's own staff list, so a stranger who signs up on their own can never see patient data, only a recognised member of the team can.



### Backed up automatically

The database backs itself up daily without anyone needing to remember to do it. If a device is ever lost, the clinic's records are not.



### Reports, never a bare link

X-rays and MRIs live in private storage. Every time one is opened, by staff or by a patient, a fresh, time-limited link is generated for that one view, then it expires.

NEW: APPENDIX · 2 OF 2

Who owns the data, and what happens if something goes wrong

15



The clinic owns everything

Every patient record, prescription and report belongs to Ramachandra Ortho Care, not to made., and not to any third party. If the clinic ever wanted to move to different software, the data is exportable, not locked in.



If something breaks

The same reliability watchdog pattern already running for Phase I extends to the new record-keeping. A fault is caught and flagged, not discovered days later when someone goes looking for a missing file.



An audit trail that can't be edited

The activity log on page 17 is append-only by design: entries can be added, never rewritten or deleted, so it stands up as a real record of who did what and when.



Built for how Indian clinics actually run

WhatsApp for reminders because that's where patients already are. Phone-number login because that's the credential nobody forgets. No assumption that every patient has an email or a smartphone app.

EVERY WORKSTREAM, ONE TABLE

Phase II deliverables

16

| WORKSTREAM              | PROPOSED DELIVERABLES                                                | STATUS   |
|-------------------------|----------------------------------------------------------------------|----------|
| A · Reception ops       | Billing action on the Today queue, real patient search & directory   | Phase II |
| B · Patient EMR         | Extended profile, care timeline, visit/prescription/report/plan tabs | Phase II |
| C · Doctor workspace    | Structured consultation form, autosave, 4 diagnosis templates        | Phase II |
| C · Prescriptions       | Medicine builder, favourites, print/PDF letterhead page              | Phase II |
| D · Reports & imaging   | Private upload, signed links, timeline linking                       | Phase II |
| E · Billing & inventory | Line-item invoicing, stock list, auto-decrement ledger               | Phase II |
| F · Patient portal      | OTP login, dashboard, follow-up reminders via WhatsApp               | Phase II |
| G · Analytics & trust   | Weekday/collections charts, staff roles, activity log                | Phase II |

**Suggested first-release focus, if a smaller starting scope is preferred:** doctor workspace, prescriptions and billing (weeks 1-5, Option B's first payment) cover the visit end to end and are the highest-impact place to start. Everything else can follow on the same timeline without re-architecting anything already shipped.

# The record catches up to the care already being given.

Phase I proved patients would use it. Phase II makes sure everything Dr. Ramachandra decides in the room is never lost, always findable, and always follows the patient home.